

Guideline

Management of acute severe headache (adults in emergency department and acute medicine)

1 Scope

Adult patients presenting to the emergency department (ED) with an acute severe headache.

2 Purpose

This guideline is to help the clinician evaluating a patient with acute severe headache.

3 Introduction

Headache patients account for between 1–2% of attendances to the ED.^[1] The primary objectives for the ED clinician are deceptively simple – make the diagnosis, relieve the headache, and investigate and manage appropriately.

Headache is defined as primary headache disorders, secondary headache and neuralgias by the International Headache Society (<http://www.ichd-3.org>)^[2], (see [Appendix 1](#)). The majority of headache is primary. Secondary headaches are those precipitated by another condition or disorder -local or systemic- and change temporally in line with that condition or disorder, i.e. they resolve when the underlying cause is treated. Serious causes of secondary headache are rare in the population as a whole but more common in the ED population, where between 2 and 5% of patients have a serious secondary cause.^[3,4]

The diagnostic priority in the ED is to identify those patients with possible secondary headache, and investigate them appropriately.

Headaches should be labelled as primary only when important secondary causes have been considered and safely excluded.

Primary Headache disorders:

- Migraine
- Tension Headache
- Cluster Headache & other trigeminal autonomic cephalgias (TACs)

Serious secondary causes to be excluded in the ED:

- Subarachnoid haemorrhage (see below)
- Carotid dissection (severe facial pain)
- Dural venous sinus thrombosis
- Idiopathic intracranial hypertension

- Intracranial tumours
- Giant cell arteritis / temporal arteritis (unlikely if age less than 50 years)
- Infective causes: bacterial meningitis/ brain abscess/ frontal sphenoidal sinusitis
- Acute angle closure glaucoma (AACG)
- Carbon monoxide poisoning

Other headache types:

Medication overuse headache

Common benign headache & other primary headaches

Neuralgia, e.g. trigeminal neuralgia.

4 Assessment of the patient with headache

The history is key to diagnosing the cause of the headache. For all the primary headache disorders, there are no useful diagnostic tests and diagnosis is based on history alone. The British Headache Society (BASH) Guidance provides an excellent overview of history taking in headache. ^[6]

4.1 Primary headache

A table summarising the typical history for common primary headaches - migraine, tension headache and cluster headache- is provided in [Appendix 2](#). In the ED, management of these should focus on pain control-see sections 8-10 below.

It is important to provide reassurance regarding the diagnosis. Many patients who attend the ED do so because they are concerned about secondary causes (such as brain tumours) and believe they require imaging. Often reassurance is all that they require to self-manage their condition.

Once the patient is pain-free, provide advice on self-management. Local detailed guidelines including patient headache diary and information leaflets can be found in the Integrated care treatment pathway: refractory headache and migraine in adults. British Association for the Study of Headache (BASH) patient headache diary and information leaflets are available at <http://www.bash.org.uk/guidelines/>. A [headache diary](#) is also available on Merlin (see also [Appendix 4](#)).

Advise GP review if symptoms do not respond to self-management.

4.2 Secondary headache

From RCT evidence, the most consistent indicators for serious secondary headache are:

- Thunderclap onset (reaches maximal intensity within 5 minutes)
- Associated focal neurological deficit
- Associated systemic symptoms
- Age >50 years.

Red flag features in the history include:

- Headache that is new or unexpected in an individual patient
- Thunderclap headache (intense headache with abrupt or “explosive” onset) • Headache with atypical aura (duration >1 hour, or including motor weakness). (For “typical” aura symptoms, see Section 7 Migraine.)
- Aura occurring for the first time in a patient during use of combined oral contraceptives
- New onset headache in a patient older than 50 years
- Persistent morning headache with nausea
- Progressive headache, worsening over weeks or longer
- Headache associated with postural change
- Headache triggered (not just made worse) by cough, sneezing, straining on defecation, exercise or intercourse / orgasm
- New onset headache in a patient with a history of cancer
- New onset headache in a patient with a history of HIV infection or immunosuppression.

Note that the following headache characteristics do not rule out serious pathology:

- Severity of headache
- Clinical features of headache
- Response to treatment.

Note: Response to therapy should **not** be used as the sole diagnostic indicator of the underlying aetiology of an acute headache

Examination: Comprehensive neurological examination is required, both to identify secondary causes, and to support a primary diagnosis. If the history supports migraine or tension headache a normal neurological examination reduces the likelihood of a secondary cause to that of the background population.^[5] Examination needs to include fundoscopy, visual fields, coordination, tone, reflexes and gait.

Useful brief ways to perform the neurological examination are available at:

<http://www.youtube.com/watch?v=wyBNYB0RLvU>

<http://www.youtube.com/watch?v=q56WgXvn0iU> (taken from BASH national headache management system.^[5])

5 Which patients require neuroimaging in the ED?

The following patient groups presenting to the ED **with headache** should undergo **emergency** non-contrast head CT scan (during their ED initial assessment) and should be kept in a monitored environment:

- Abnormal findings in a neurological examination (i.e. focal deficit, altered mental status, altered cognitive function)
- Acute sudden-onset/ thunderclap headache
- Severe headache with syncope or seizure
- Severe incapacitating headache after head injury, as per NICE Guidance.^[7]

The following patient groups presenting to the ED **with headache** should be considered for **urgent** non-contrast head CT scan (during their ED assessment):

- Suspicion of raised ICP (provocative factors such as coughing, bending, Valsalva)
- Human immunodeficiency virus (HIV)-positive/ immunosuppressed patients with a new type of headache
- Progressive headache (especially if associated with vomiting)

There should be a strong presumption in favour of conducting CT scanning among patients referred for exclusion of SAH by their GP. Additional consideration in favour of CT scanning should also be given to older patients.

6 Ruling out sub-arachnoid haemorrhage (SAH) in thunderclap headache

Patients with a thunderclap headache who have negative findings in a head CT scan, normal opening pressure (6-20 cm H₂O) , and negative findings in cerebrospinal fluid (CSF) analysis (minimum 12 hours post onset) can be discharged from the ED with follow up arranged with their general practitioner (GP) or neurologist.

No patient with sudden onset severe headache (first or worst) should be discharged without exclusion of SAH.

The sensitivity of lumbar puncture for subarachnoid blood is 100% from 12 to 72 hours post onset of headache, and this can be considered to definitively exclude subarachnoid haemorrhage.

The sensitivity for SAH of a CT head scan performed within six hours of a thunderclap headache onset is very close to 100%^[5,10]. The CT should ideally be reported by a Consultant Neuroradiologist to optimise its sensitivity-and this is usually routinely available within 24 hrs on working days.

If the CT head is done within six hours and is normal, a lumbar puncture will only identify a SAH in 1 to 2 per 1000 cases. It is reasonable to consider the merits of whether proceeding to a lumbar puncture is necessary when the patient is reluctant to undergo this procedure and is confirmed CT negative (as reported by Consultant Neuroradiologist) within 6 hrs of onset and where all of the following apply:

- a) headache did not occur in context of trauma
- b) headache is nearly resolved
- c) headache features do not really fit SAH i.e. not truly thunderclap
- d) patient has no neurologic deficit and is alert
- e) patient has no prior personal or family history of intra-cranial aneurysms and/or SAH; has no personal history of brain tumours
- f) patient does not have any of the high risk variables using the following Ottawa Rule:

1. Age ≥ 40 y
2. Neck pain or stiffness
3. Witnessed loss of consciousness
4. Onset during exertion
5. Thunderclap headache (instantly peaking pain)
6. Limited neck flexion on examination

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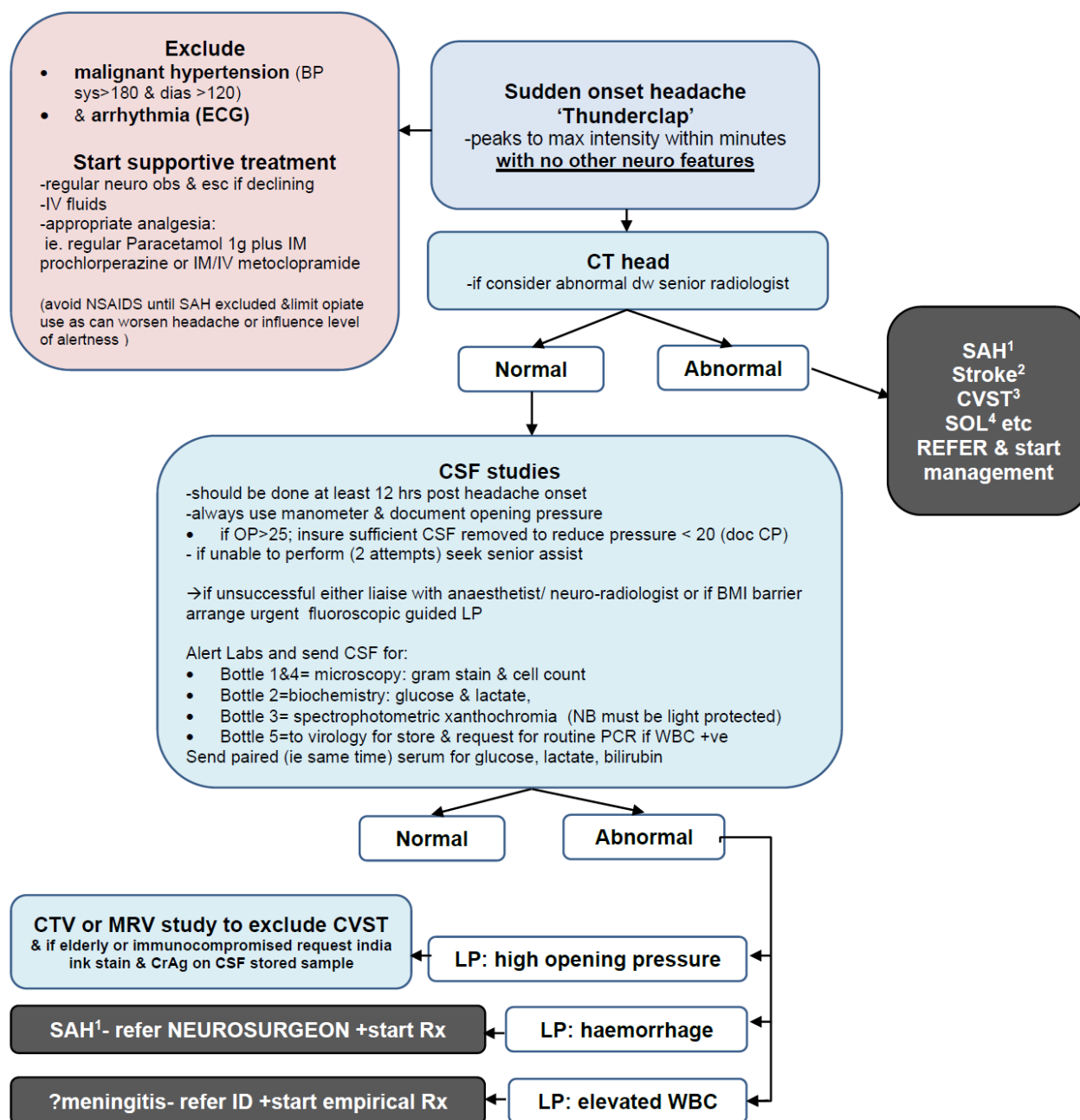
You should discuss the risks and benefits of lumbar puncture with your patient and assist them reach a decision as to whether or not they would like to proceed.

After 6 hours the sensitivity of CT falls to <90%. Lumbar puncture should therefore be considered in all patients with a normal CT scan performed >6 hours after onset and a history suggestive of SAH. Lumbar puncture should be delayed until a minimum of 12 hours (and remains useful up to 2 weeks) post headache onset, to optimise sensitivity for detection of bilirubin (spectrophotometric assay which requires sample to be light protected).

Patients with CT scans or lumbar puncture results consistent with subarachnoid haemorrhage should be referred directly to the on-call neurosurgical specialist registrar (SpR).

Infogram 1: Immediate investigation in sudden onset/ thunderclap headache (courtesy of Dr Jane M Anderson, Consultant Neurologist)

Immediate investigation in sudden onset/ thunderclap headache_Dr Jane M Anderson



- ¹SAH: contact neurosurgery on-call; start IV fluids+++, nimodipine 60mg 4 hourly, high level neuro & cardiac monitoring; arrange CTa if stable; re-CT if any change in alertness or focal signs to exclude hydrocephalus or sequelae of vasospasm
 - ²Stroke: contact stroke team for advice and possible transfer of care; if ischaemic load 300mg aspirin or 150mg clopidogrel
 - ³CVST: start anticoagulation unless contraindication with treatment dose LMWH pending warfarin loading; monitor for min 24 hrs for seizures or focal deficit including altered alertness; discuss any concerns with neuro on call & refer for inpatient neuro rev
 - ⁴SOL: contact neurosurgery on-call if concern re clinical state, significant oedema, or likely primary CNS tumour & take advice as to indication for dex; arrange urgent CTCAP; involve medical team and oncology
- WITH ALL: repeat-CT if any change in alertness or new focal signs

CVST- cerebral venous sinus thrombosis;
RCVS- reversible cerebral vasoconstriction syndrome
SOL- space occupying lesion

Dissection- cervical arterial dissection
SAH- subarachnoid haemorrhage

IIH- idiopathic intracranial hypertension
SIH- spontaneous intracranial hypotension

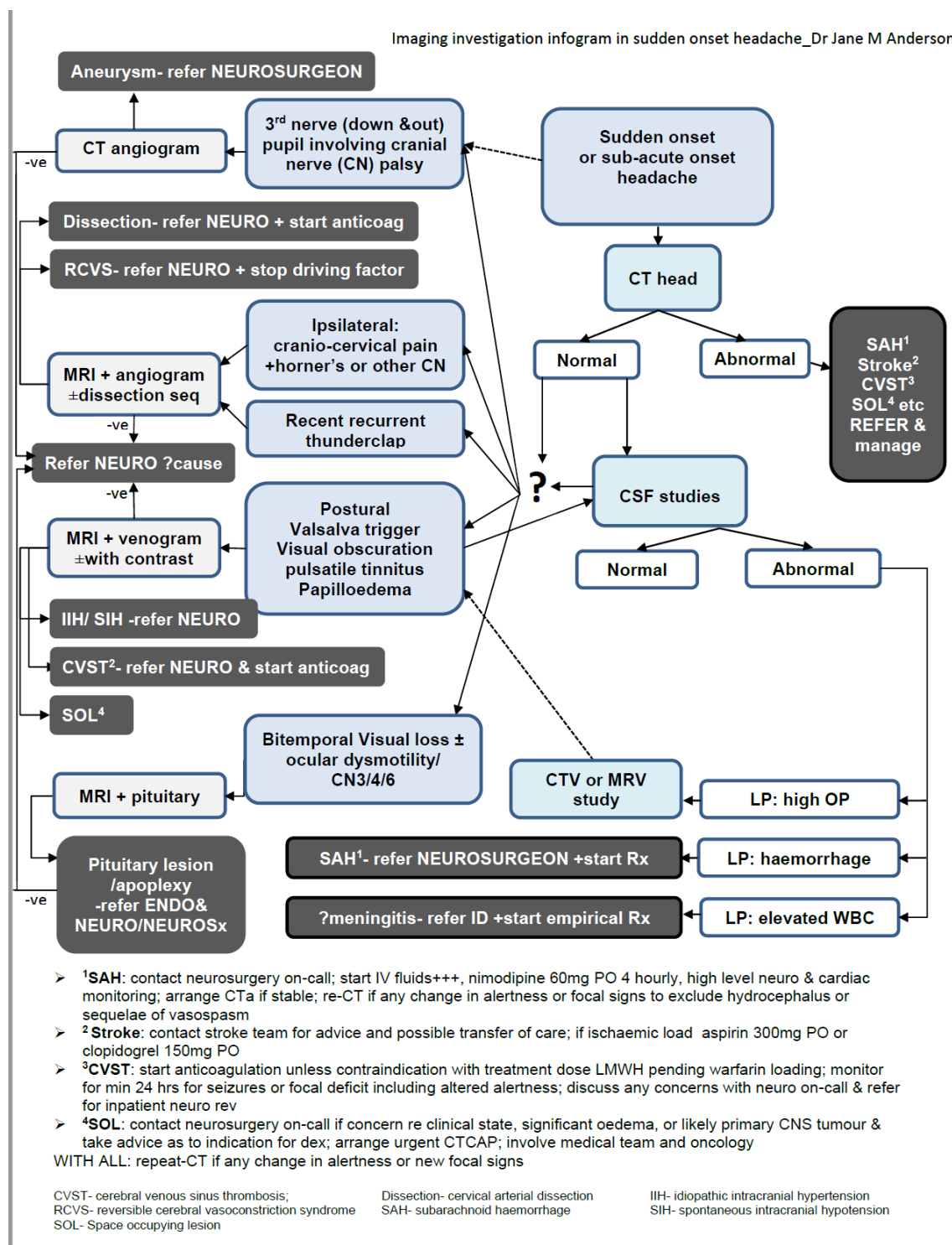
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Thunderclap headache has been associated with other secondary pathologies including: cerebral venous sinus thrombosis, malignant hypertension and arterial dissection, cerebrovascular causes such as intracranial haemorrhage, infective causes, non-vascular neurological causes (such as pituitary apoplexy) and spontaneous intracranial hypertension. Spontaneous intracranial hypotension classically presents as an orthostatic headache.

If CT and CSF are normal, additional investigations may be indicated if warranted by the clinical presentation.

Infogram 2: Imaging investigations in sudden onset headache (courtesy of Dr Jane M Anderson, Consultant Neurologist)



Which headache patients need a head CT prior to performing lumbar puncture?

The majority of patients presenting to the ED with headache will require a CT head. However occasionally there may be an urgent need to perform LP prior to head CT. Adult patients with headache who have signs of increased intracranial pressure (including papilloedema, absent venous pulsations on fundoscopy, altered mental status, or focal neurological deficits) must undergo a CT head before having an LP.

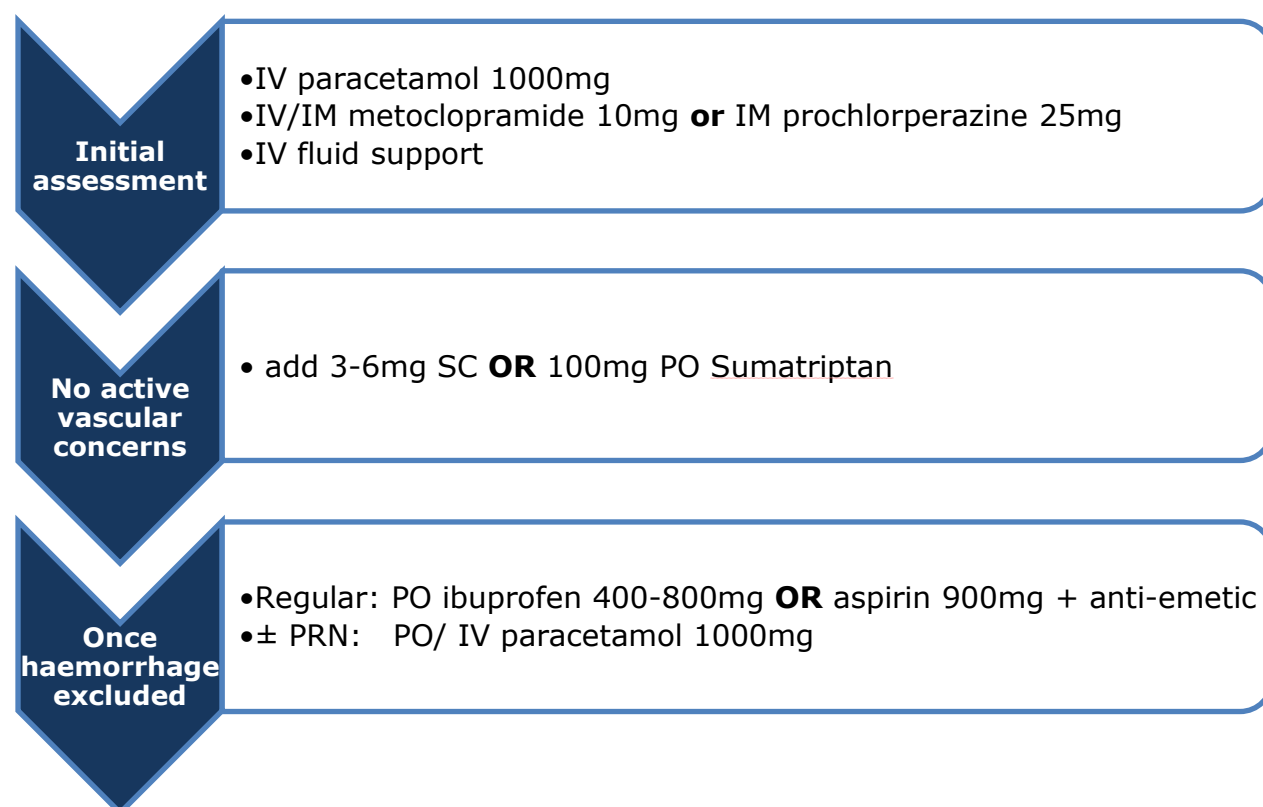
In the absence of findings suggestive of increased intracranial pressure, an LP can be performed without obtaining a CT head (note: An LP does not assess for all causes of a sudden severe headache.)

Indications for lumbar puncture in patients with headache:

- Fever (meningitis or encephalitis suspected)
- Suspected subarachnoid haemorrhage

7 Acute sudden onset headache management in the ED

As the majority of headache will still be primary headache, the initial treatment approach should be appropriate to this:



Avoid opioids where possible as the above options are more effective in migraine and opioids are associated with worsening of associated symptoms,

longer hospital admissions and an increased risk of patients developing medication overuse headache.

Be cautious using triptans in the following groups:

- Age > 65 years
- Active vascular concerns - suspected subarachnoid hemorrhage, suspected cerebral venous sinus thrombosis, suspected cervical artery dissection, suspected giant cell arteritis or other central vasculitis, uncontrolled hypertension, peripheral vascular disease, ischaemic heart disease/ heart failure, previous myocardial infarction, previous TIA/stroke

8 Migraine: diagnosis and management

8.1 Diagnosis

The NICE^[8] simplified diagnostic criteria for migraine in adults are:

Repeated attacks of headache lasting 4-72 hours which have these features:

At least two of:	At least one of:
• unilateral pain • throbbing pain • aggravation by movement • moderate or severe intensity	• nausea/ vomiting • photophobia • phonophobia

One third of migraine patients experience aura. A typical aura comprises of fully reversible positive visual and/or sensory/ and/or speech symptoms, evolving over minutes with a total duration of up to 60 minutes. Typically aura occurs before the headache, but may occur during or afterwards. Aura without headache can also occur and is more common in older patients.

Chronic migraine is migraine that occurs on at least 15 days per month for at least 3 consecutive months. It affects 2% of patients. Two thirds of patients with migraine will also have medication overuse.

8.2 Treatment of acute migraine

- Triptan (5HT₁- receptor agonist)
 - Oral sumatriptan 50mg-100mg or patients own triptan supply if not already used and emesis not limiting oral route.
 - If rapid onset or emesis limiting, consider subcutaneous sumatriptan 3-6mg. 3mg may be sufficient to resolve symptoms and gives reduced incidence of side effects.
 - a second dose can be given after 2h if the migraine recurs
 - maximum dose dependent on triptan with subcut sumatriptan 12mg/24h, oral sumatriptan 300mg/24hr

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- give in combination with NSAID or paracetamol **and** an anti-emetic. Combination with a long acting NSAID such as naproxen and an anti-emetic increases efficacy.
- Simple analgesia (adequate dose combined with triptan)
 - Dispersible aspirin 900mg **or** ibuprofen 400 – 800mg **or**
 - paracetamol 1g
- Anti-emetic¹
 - prochlorperazine 25mg IM **or**
 - metaclopramide 10mg IV (contraindicated in women under 21 years)
- Rehydration
 - sodium chloride 0.9% IV

Do not give opioids as the above options are more effective and opioids are associated with worsening of migrainous associated symptoms, longer hospital admissions and an increased risk of patients developing medication overuse headache.

To avoid medication overuse headache it is important that the target set for use of analgesics is well below the threshold hence ≤ 10 days per month and of these ≤ 6 days per month can be triptan based (if codeine or opioids must be used then ≤ 6 days per month can be triptan/codeine/opioid based).

Give every patient with migraine a headache diary on discharge -see [Appendix 4](#).

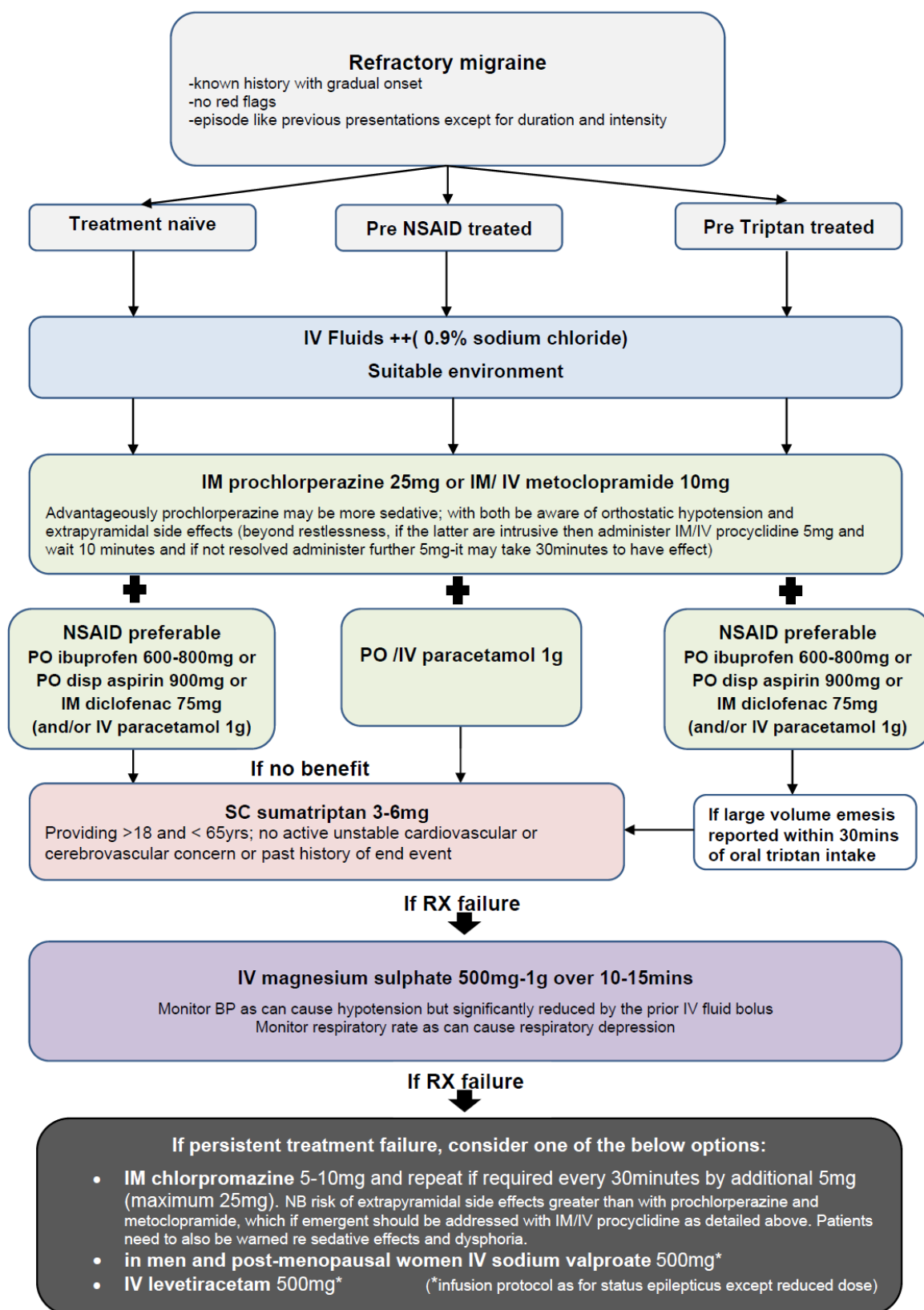
If a patient with migraine requiring A&E attendance responds well to treatment but continues with low grade pain then consider naproxen bridge with appropriate gastric protection on discharge from hospital (providing no contra-indication or renal compromise) -using naproxen 500mg bd for 2 weeks then reducing to 250mg bd for 2 weeks then 250mg od for one week then stop.

¹ Alternative anti-emetics such as ondansetron and cyclizine can be used when nausea/emesis is insufficiently controlled, or where tolerability is an issue

Infogram 3: Management of refractory migraine presenting to Emergency / Acute department¹

(courtesy of Dr Jane M Anderson, Consultant Neurologist)

Management of refractory migraine in the emergency/ acute department_Dr Jane M Anderson



8.3 Prevention

- Preventative treatment should be considered for patients with 5 or more limiting migraine days per month.
- Advise follow up with GP -with onward referral to neurologist if patient fails to respond to more than 3 different preventatives taken at the minimal effective dose for at least 3 months (refer to Integrated care treatment pathway: refractory headache and migraine in adults).
- A headache diary is essential to monitor response to treatment and may help identify triggers.
- Avoid known triggers
- 1st line prophylaxis options include: propranolol, topiramate and amitriptyline. (Topiramate carries a risk of fetal malformation and may reduce the effectiveness of hormonal contraceptives -this needs to be discussed with all women of child-bearing age). In general, if starting these medications in the ED try to discuss this with the on call Neurology Registrar, and if starting them ensure that the GP is aware and will follow up the patient.
- Locally agreed patient headache diary and information leaflets are available in the Integrated care treatment pathway: refractory headache and migraine in adults
- British Association for the Study of Headache (BASH) patient headache diary and information leaflets are available at <http://www.bash.org.uk/guidelines/>

9 Cluster headache -diagnosis & management

9.1 Diagnosis

Cluster headache is one of the Trigeminal Autonomic Cephalgias (TACs). This is a group of 4 types of headache with prominent autonomic features (and similar pathophysiology). The group comprises cluster headache, paroxysmal hemicrania, SUNA/SUNCT and hemicrania continua. All are rare. Clinical features and management are reviewed by BASH^[5].

Cluster headache is the commonest TAC, with an incidence of 0.1%. Peak onset is in the 30s and 40s and it is commoner in men (4:1 M:F) and in smokers.

Attacks are classically unilateral (though the affected side can vary) and associated with autonomic features on the painful side, eg lacrimation, facial sweating or flushing, nasal congestion, rhinorrhoea, ear fullness, eyelid swelling, ptosis and miosis. Attacks typically last 15 minutes-2 hours and may occur 1-2 times a day, typically at night waking patient from sleep. During attacks patients are typically restless and lack motion-sensitivity, with attacks starting and stopping abruptly – both these features help differentiate this condition from migraine.

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Attacks generally occur in clusters lasting up to 3 months. However 20% of patients develop near daily chronic cluster headache lasting >1 year with less than 1 month in that year in remission.

See [Appendix 3](#) for comparison of diagnostic features for cluster headache vs other primary headache.

9.2 Acute treatment

- High flow oxygen 100% at least 12 litres/minute (via a non-rebreather mask) for 15-20 minutes,
- Sumatriptan 6mg subcutaneous injection, maximum 2 doses/24 hours.

9.3 Prevention

Verapamil is an effective preventive treatment in cluster headache. It should be initiated at a dose of 120mg BD, with increase by 120mg every 2 weeks to a TDS regime up to a maximum of 960mg daily. ECG should be carried out at baseline and after each increase in dose. Do not start this in the ED without discussion with the GP or Neurologist, and if starting it ensure that the GP is aware and will follow up the patient (refer to Integrated care treatment pathway: refractory headache and migraine in adults-Verapamil patient information leaflet).

Prednisolone 60mg OD taken for 5 days can be used for patients who have multiple daily attacks as cover whilst preventative treatment is being initiated. The dose will need to be tapered to stop once the course is complete by reducing by 10mg every 72 hours to stop. No more than two courses should be given in a 12 month period.

Advise patient to avoid alcohol during a bout of attacks as can trigger further attacks.

Discuss with GP regarding arranging provision of home oxygen if patient has >2 attacks per 24hrs or if subcutaneous sumatriptan not sufficiently effective or tolerated.

10 Tension-Type Headache (TTH) – ED Management

10.1 Diagnosis

This is the most common primary headache disorder, with a lifetime prevalence of 42%. The headache is typically not severe, however patients may present to the ED if they are anxious over a sinister underlying cause.

TTH is characterised by mild-moderate and not severe, headache. It is bilateral and often described as pressing or tightening like a vice or tight band. It typically lacks the specific features that characterise migraine such as nausea, light and noise sensitivity. The headache is not aggravated by routine physical activity and this is a key criterion for diagnosis. Duration of pain can be variable with a

range from half an hour to several days. TTH on 15 or more days per month for at least 3 months is termed chronic TTH.

See [Appendix 3](#) for a comparison of TTH and other primary headache types.

10.2 Treatment

Avoid analgesics unless compromising function. Options are:

Paracetamol 1g QDS

OR

Aspirin 300-900mg QDS (max 4000mg daily)

Do not offer opioids.

To avoid medication overuse headache it is important that the target set for use of analgesics is well below the threshold hence ≤ 10 days per month.

10.3 Prevention

Amitriptyline 10mg ON. Increase by 10-25mg to a maximum of 100-150mg at night.

11. Monitoring compliance with and the effectiveness of the guideline

The use and effectiveness of this guideline will be monitored, continuously through the following processes:

- clinical incident review
- clinician feedback
- patient complaints.

Results will be fed back to the appropriate clinicians, who will consider review, and implement identified actions, as appropriate.

12. Appendices

1. Classification of Headache Disorders

(The International Classification of headache disorders, 3rd Edition, simplified table).

Primary headaches	1. Migraine, <i>including</i>: 1.1 Migraine without aura 1.2 Migraine with aura 2. Tension-type headache, <i>including</i>: 2.1 Infrequent episodic tension-type headache 2.2 Frequent episodic tension-type headache 2.3 Chronic tension-type headache	3. Cluster headache and other trigeminal autonomic cephalalgias, <i>including</i>: 3.1 Cluster headache 4. Other primary headaches
Secondary headaches	5. Headache attributed to head and/or neck trauma, <i>including</i>: 5.2 Chronic post-traumatic headache 6. Headache attributed to cranial or cervical vascular disorder, <i>including</i>: 6.2.2 Headache attributed to subarachnoid haemorrhage 6.4.1 Headache attributed to giant cell arteritis 7. Headache attributed to non-vascular intracranial disorder, <i>including</i>: 7.1.1 Headache attributed to idiopathic intracranial hypertension 7.4 Headache attributed to intracranial neoplasm 8. Headache attributed to a substance or its withdrawal, <i>including</i>: 8.1.3 Carbon monoxide-induced headache 8.1.4 Alcohol-induced headache	8.2 Medication-overuse headache 8.2.1 Ergotamine-overuse headache 8.2.2 Triptan-overuse headache 8.2.3 Analgesic-overuse headache 9. Headache attributed to infection, <i>including</i>: 9.1 Headache attributed to intracranial infection 10. Headache attributed to disorder of homeostasis 11. Headache or facial pain attributed to disorder of cranium, neck, eyes, ears, nose, sinuses, teeth, mouth or other facial or cranial structures, <i>including</i>: 11.2.1 Cervicogenic headache 11.3.1 Headache attributed to acute glaucoma 12. Headache attributed to psychiatric disorder
Neuralgias and other headaches	13. Cranial neuralgias, central and primary facial pain and other headaches, <i>including</i>: 13.1 Trigeminal neuralgia	14. Other headache, cranial neuralgia, central or primary facial pain

**This table is a simplification of the IHS classification*

2. History taking in headache

1. How many different headache types does the patient experience?

Separate histories are necessary for each. It is reasonable to concentrate on the most bothersome to the patient but others should always attract some enquiry in case they are clinically important.

2. Time questions	a) Why consulting now? b) How recent in onset? c) How frequent, and what temporal pattern (especially distinguishing between episodic and daily or unremitting)? d) How long lasting?
3. Character questions	a) Intensity of pain b) Nature and quality of pain c) Site and spread of pain d) Associated symptoms
4. Cause questions	a) Predisposing and/or trigger factors b) Aggravating and/or relieving factors c) Family history of similar headache
5. Response questions	a) What does the patient do during the headache? b) How much is activity (function) limited or prevented? c) What medication has been and is used, and in what manner?
6. State of health between attacks	a) Completely well, or residual or persisting symptoms? b) Concerns, anxieties, fears about recurrent attacks, and/or their cause

3. Diagnostic features of common primary headache

Diagnosis of tension-type headache, migraine and cluster headache

Headache feature	Tension-type headache		Migraine (with or without aura)		Cluster headache	
Pain location ¹	Bilateral		Unilateral or bilateral		Unilateral (around the eye, above the eye and along the side of the head/face)	
Pain quality	Pressing/tightening (non-pulsating)		Pulsating (throbbing or banging in young people aged 12–17 years)		Variable (can be sharp, boring, burning, throbbing or tightening)	
Pain intensity	Mild or moderate		Moderate or severe		Severe or very severe	
Effect on activities	Not aggravated by routine activities of daily living		Aggravated by, or causes avoidance of, routine activities of daily living		Restlessness or agitation	
Other symptoms	None		- Unusual sensitivity to light and/or sound or nausea and/or vomiting. - Aura: symptoms can occur with or without headache and; are fully reversible, develop over at least 5 minutes, last 5 - 60 minutes. Typical aura symptoms include visual symptoms such as flickering lights, spots or lines and/or partial loss of vision; sensory symptoms such as numbness and/or pins and needles; and/or speech disturbance.		On the same side as the headache: - Red and/or watery eye - Nasal congestion and/or runny nose - Swollen eyelid - Forehead and facial sweating - Constricted pupil and/or drooping eyelid	
Duration of headache	30 minutes–continuous		4–72 hours in adults 1–72 hours in young people aged 12–17 years		15–180 minutes	
Frequency of headache	< 15 days per month	≥ 15 days per month for more than 3 months	< 15 days per month	≥ 15 days per month for more than 3 months	1 every other day to 8 per day ³ , with remission ⁴ >1 month	1 every other day to 8 per day ³ with a continuous remission ⁴ <1 month in a 12-month period
Diagnosis	Episodic tension-type headache	Chronic tension-type headache ²	Episodic migraine (with or without aura)	Chronic migraine (with or without aura)	Episodic cluster headache	Chronic cluster headache

¹ Headache pain can be felt in the head, face or neck. ² Chronic migraine and chronic tension-type headache commonly overlap. If there are any features of migraine, diagnose chronic migraine. ³ Frequency of recurrent headaches during a cluster headache bout. ⁴ The pain-free period between cluster headache bouts.

Adapted from 'Headaches' (NICE clinical guideline 150), available from www.nice.org.uk/CG150

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4. Headache diary -to be given to all patients with migraine

Link to diary: <http://merlin/Lists/DMSRecords/DispRecordTabsDoc.aspx?ID=23355>

Sample diary:

Month:

Patient Name & dob/HN

detailed headache diary_migraine_patient held record

DETAILED HEADACHE DIARY_MIGRAINE

Please fill in the diary below on a day by day basis and bring it with you to your headache clinic appointment.

This page illustrates how the diary should be filled out.

In the date column please indicate if menstrual period (P) and highlight weekends (circle the date)

Please estimate how many hours of headache you experience that day

The headache is scored as headache free(NONE), mild on a scale 1-4(GREEN), moderate on a scale 5-7 (AMBER) or severe on a scale of 8-10(RED) –please mark with an X in the appropriate column

Please also mark with an X any associated migrainous features including aura, nausea or brain fog, and avoidance of light/ noise/moving due to sensitivity

Mark X in the acute painkillers column if you treat any type of pain (not just head pain); please circle the X if you use a triptan, codeine, tramadol, opioid

Note any change to your regular preventive/ prophylactic headache medication

At end of month please total the number of days an X is marked at the bottom of each pain column and the acute painkiller column

Date	headache duration (hrs)	NONE Headache free	GREEN Mild				AMBER Moderate			RED Severe			Aura	queasiness nausea vomiting	Brain fog	Tend to avoid			Acute painkillers
			1	2	3	4	5	6	7	8	9	10				Light	Noise	Moving	
1		X																	
2							X						X	X		X	X	X	
3P										X				X	X	X	X	X	Aspirin + Rizatriptan
4P												X		X	X	X	X	X	Aspirin+ Rizatriptan
5P												X		X	X	X	X	X	Aspirin
6P										X				X	X			X	
7				X										X				X	
8		X																	
Total number			=2	=0	=1	=0	=0	=1	=0	=0	=1	=1	=2						= 3 (triptan 2)

13. References

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Emergency department

Emergency and perioperative care directorate

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